

شعار الجمهورية

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Name: امنيه عبدالمنعم ابراهيم ابراهيم National ID: 29005268800647	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



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IMMUNOHISTOCHEMISTRY REPORT

Name: أمينة عبد المنعم ابراهيم
PATIENT'S I.D :29005268800647
Age: 32 Y.

TM: 64 /2023

Specimen Received Data: 22 / 02 /2023
Date Report Issued: 26/ 02 /2023

CLINICAL DATA: A case of *right* breast cancer, for estrogen, progesterone receptors, Her-2/neu and Ki-67.

PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana IView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

RESULTS:

Examination of section treated for estrogen receptors revealed moderate positive nuclear staining in 40% of tumor cells by chromogen.

Examination of section treated for progesterone receptors revealed moderate positive nuclear staining in 33% of tumor cells by chromogen.

Examination of section treated for Her 2/neu revealed moderate membrane staining in >10% of tumor cells by chromogen (score 2+).

Examination of section treated for Ki67 revealed positive nuclear stain in average of ±60% of tumor cells.

CONCLUSION:

Tumor Cells Were:

- > **Positive** For Estrogen Receptors, Score 6/8.
- > **Positive** For Progesterone Receptors, Score 6/8.
- > **Equivocal** For Her-2/neu (Score 2+).
- > 60% Proliferative index

COMMENT: All controls show appropriate reactivity.

DR. AYA MAGDY
DR. YOUSRA RAAFAT

Signature of Dr. Yousra RAAFAT



IN SITU HYDRIDIZATION REPORT

NAME: أمينة عبد المنعم ابراهيم
PATIENT'S I.D.: 29005268800647
AGE: 32 Y.

TM : 64 S /2023

Specimen Received Data: 01 / 03 /2023
Date Report Issued: 04/ 03 /2023

CLINICAL DATA: A case of *right* breast cancer, for detection of HER2 amplification by dual in situ hybridization (DISH).

PROCEDURE: Tissue sections from paraffin block were used for quantitative detection of HER2 amplification using Ventana Benchmark Ultra System autostainer & VENTANA INFORM HER2 Dual DNA Probe Cocktail Assay.

RESULTS:

The HER2 NEU/ CEP 17 ratio was 6
HER2 copies were 9.9

COMMENT:

HER2 Status by DiSH: **AMPLIFIED**

DR. PASSANT ESSAM SHIBEL
DR. AYA MAGDY

A. Magdy

Passant

Patient name: OMNYA ABD ELMONAEM IBRAHEM	Age: 32 Years
Modality Name: CT	Study Date: 21/02/2023 12:38:43 ,
Patient id: 800647	

GIT IMAGING UNIT

POST-CONTRAST CT EXAMINATION OF ABDOMEN & PELVIS REVEALED:

- The liver is average size, showing smooth outline and homogenous parenchymal texture with no pathologically enhancing focal lesion. No dilated intra hepatic biliary radicles.
- Normal CT appearance of the spleen, pancreas, suprarenal glands IVC and aorta.
- Normal function and morphology of both kidneys.
- Prominent subcentimetric para-aortic, aortocaval and mesenteric lymph nodes, likely reactive.
- Normal urinary bladder and pelvic viscera.
- No ascites or pelvic collection.
- IUD is noted in situ.
- Small fat containing umbilical hernia.
- Bilateral sacroilitis, more evident on the left side.

OPINION:-

- No CT evidence of pelviabdominal metastatic deposits.

Dr. Marie N. Grace, MD

Res. Nema Aboelmagd

Patient Name: *Omneya Abd El-Monim Ibrahim*

Patient ID: *577354*

Exam Date: *16/3/2024*

^{99m}Tc -MDP WHOLE BODY BONE SCAN

Desired information:-

- skeletal survey.

Technique:

- anterior & posterior whole body e bone scan performed 2 hours following IV administration of 20 mCi $\text{Tc-}^{99m}\text{MDP}$.

Report:-

- No hot spot that may account for osseous deposits could be detected all over the rest of the surveyed skeleton.
- Degenerative changes at dorso-lumbar vertebrae, more at mid lumbar for follow up.
- Both kidneys are visualized.

Conclusion:

- *The surveyed skeleton is free of osseous deposits.*
- *Follow up bone scan is advised.*

Thanks;

Nuclear Dr. Huda Fathy, MD



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مدر

MDT FORM

15/3/2023 GA

Name	انيس عبد الحليم	MRN	٢٠٢٢/٩٦٦
Age	32	ID	٢٩٠٠٥٦٧٨٩٠٦٧
Menopausal status		Phone	٠١١٤٧١٩٤٩٧٢
NRS SCORE		Marital status	m.
BMI		Family history	
CO morbidity		REASON OF MDT	Nov

Rt Breast.				Clinical Data	
BX: IDC/LC					
ER: 6/8	PR: 6/8	HER 2: +2 SSH amp.	Ki67: 60%.		
Sonomammography: Rt UOQ 11 clock 3.2x3.8 + LN. CE SM unifocal					
Mets Workup: Pst					

Recommendations:

Feibroid - clip inserted

① Bone scan

② Echo

③ Lab

MDT OUTCOME & PLAN

Start ① Tamoxifen (ovarian protection) ② NAT (TCR) with regular clinical assessment ③ Re-do mammography post NAT the candidate Rt. UOQ 3.2x3.8.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
D. F. ٢٠٢٢/٤/١٥					